

August 12, 2026

South Carolina Medicaid (Healthy Connections / SCDHHS)
Provider Appeals Department

RE: Formal Appeal of Claim Denial — Jeffery Daniel (MRN MUSC2463868), Claim MUSC-20260203-FA06-1

Patient	Jeffery Daniel (DOB 1920-04-23)
MRN	MUSC2463868
Member ID / Group	SCD993026943 / GRP-94226
Claim number	MUSC-20260203-FA06-1
Date of service	2026-02-03
Procedure billed	CPT 99214 — Office/outpatient visit, established patient, moderate complexity
Primary diagnosis	R69 — Illness, unspecified
Denied amount	\$577.53
Denial code	CARC 50 / RARC N115 — These are non-covered services because this is not deemed a 'medical necessity' by the payer.
Appeal deadline	2026-04-16

Dear Appeals Review Committee:

SUMMARY OF APPEAL

MUSC Health submits this formal first-level appeal on behalf of patient Jeffery Daniel (MRN: MUSC2463868, Member ID: SCD993026943) regarding the denial of Claim MUSC-20260203-FA06-1 for a 99214 established-patient office visit rendered on February 3, 2026, by Marcus T. Bledsoe, DO (NPI 1770615243) at MUSC Health Ashley River Tower. South Carolina Medicaid (Healthy Connections / SCDHHS) denied the full billed amount of \$577.53 on March 17, 2026, citing CARC 50 ("These are non-covered services because this is not deemed a 'medical necessity' by the payer") and RARC N115 ("This decision was based on a Local Coverage Determination (LCD)"), with the accompanying remark that "Documentation submitted does not establish that the service was reasonable and necessary for the diagnosis reported." MUSC Health respectfully disagrees with this determination and requests reversal and payment at the allowed amount of \$313.89.

CLINICAL BACKGROUND

Mr. Daniel is a 106-year-old male with an extensive and active chronic disease burden managed longitudinally at MUSC Health. His active conditions include atrial fibrillation, a history of stroke, type 2 diabetes mellitus, anemia, osteoarthritis of the knee, hyperglycemia, metabolic syndrome, and hypertriglyceridemia. His medication regimen reflects the complexity of this burden: he carries active prescriptions for digoxin, verapamil hydrochloride, warfarin sodium, and metformin hydrochloride, each requiring ongoing clinical monitoring for therapeutic efficacy, drug interactions, and adverse effects. His history further includes multiple electrical cardioversions for atrial fibrillation and a percutaneous mechanical thrombectomy, underscoring the severity and recurrence of his cardiac disease. A previously recorded estimated glomerular filtration rate of 19.54 mL/min/1.73m² indicates significantly reduced renal function, which directly affects the safe dosing and monitoring of several of his active medications. This clinical profile represents a patient whose ongoing management requires regular, substantive physician evaluation.

BASIS FOR APPEAL

The denial rests on the assertion that the documentation does not establish medical necessity for the service as billed under the primary diagnosis code R69. MUSC Health acknowledges that R69, as a nonspecific illness code, warrants scrutiny; however, the denial does not account for the full clinical context documented in Mr. Daniel's record. CPT code 99214 is appropriately assigned to an established-patient visit involving moderate medical decision-making or a medically appropriate history and examination of moderate complexity. Given Mr. Daniel's constellation of active chronic conditions — atrial fibrillation on anticoagulation and rate-control therapy, prior stroke, insulin-requiring diabetes with documented renal impairment, and anemia — a visit involving the management of even one of these conditions independently satisfies the threshold for moderate complexity medical decision-making under current CPT evaluation and management guidelines. The rendering provider, Dr. Bledsoe, evaluated an established patient whose polypharmacy and comorbidities require regular clinical reassessment to prevent adverse outcomes. The level of service billed is consistent with, and supported by, the documented medical history. To the extent the payer's LCD requires more specific diagnosis coding, MUSC Health is prepared to supplement the record with additional clinical documentation from the February 3, 2026 encounter upon request, and respectfully asks that the payer identify with specificity any documentation deficiency rather than issue a blanket medical necessity denial.

REQUESTED ACTION

MUSC Health requests that South Carolina Medicaid (Healthy Connections / SCDHHS) reverse the denial of Claim MUSC-20260203-FA06-1 and issue payment at the previously determined allowed amount of \$313.89 for the 99214 service rendered on February 3, 2026. Should the payer require additional clinical documentation to support this determination, MUSC Health will provide it promptly upon notification. This appeal is submitted within the 30-day appeal window, with the deadline of April 16, 2026, and is directed to the Division of Appeals and Hearings in accordance with the plan's stated appeal procedures.

Respectfully submitted,

Angela R. Whitfield, RN, BSN, CPC

Senior Appeals Specialist, Revenue Cycle Management
MUSC Health — Revenue Cycle / Patient Financial Services
(843) 792-2300 | muschealth.org

Enclosures

- Remittance advice / explanation of payment
- Itemized claim detail (UB-04 / CMS-1500)
- Relevant clinical documentation from the medical record